

LEGISLATURE OF NEBRASKA
ONE HUNDRED NINTH LEGISLATURE
SECOND SESSION

LEGISLATIVE BILL 949

Introduced by Ballard, 21.

Read first time January 09, 2026

Committee:

- 1 A BILL FOR AN ACT relating to health information; to amend sections
- 2 81-6,124, 81-6,125, 81-6,127, and 81-6,128, Reissue Revised Statutes
- 3 of Nebraska, and sections 68-2106, 71-2454, and 71-2455, Revised
- 4 Statutes Cumulative Supplement, 2024; to change provisions relating
- 5 to the prescription drug monitoring program, the statewide health
- 6 information exchange, and the Health Information Technology Board;
- 7 to harmonize provisions; and to repeal the original sections.
- 8 Be it enacted by the people of the State of Nebraska,

1 **Section 1.** Section 68-2106, Revised Statutes Cumulative Supplement,
2 2024, is amended to read:

3 68-2106 (1) The Hospital Quality Assurance and Access Assessment
4 Fund is created. Interest earned on the fund shall be credited to the
5 fund. Any money in the fund available for investment shall be invested by
6 the state investment officer pursuant to the Nebraska Capital Expansion
7 Act and the Nebraska State Funds Investment Act.

8 (2) The department shall use the Hospital Quality Assurance and
9 Access Assessment Fund, including the matching federal financial
10 participation, for the purpose of enhancing rates paid to hospitals under
11 the medical assistance program except as allowed by subsection (3) of
12 this section. Money in the fund shall not be used to replace or offset
13 existing state funds paid to hospitals for providing services under the
14 medical assistance program.

15 (3) The Hospital Quality Assurance and Access Assessment Fund shall
16 also be used to:

17 (a) Reimburse the General Fund the amount of the first quarterly
18 payment on or before June 30 of each fiscal year;

19 (b) Reimburse the department an administrative fee of three percent
20 of the assessment, not to exceed fifteen million dollars per year, to
21 collect assessments and administer directed-payment programs established
22 by the Hospital Quality Assurance and Access Assessment Act;

23 (c) Provide the Nebraska Center for Nursing Board one-half of one
24 percent of the assessment, not to exceed two million five hundred
25 thousand dollars per year, for the expansion of clinical nursing training
26 sites as authorized in subsection (3) of section 71-1798; and

27 (d) Provide funding of three and one-half percent of the assessment,
28 not to exceed seventeen million five hundred thousand dollars per year,
29 for rates for nonhospital providers in the medical assistance program,
30 continuous eligibility for children, or the designated health information
31 exchange as defined ~~authorized~~ in section 81-6,124 ~~81-6,125~~.

1 (4) In calculating rates, the proceeds from assessments and federal
2 match not utilized under subsection (3) of this section shall be used to
3 enhance rates for hospital inpatient and outpatient services in addition
4 to any funds appropriated by the Legislature.

5 (5) The department shall collect data for revenue, discharge, and
6 inpatient days from a hospital that does not file an annual medicare cost
7 report. At the request of the department, a hospital that does not file
8 an annual medicare cost report shall submit such requested data to the
9 department.

10 (6) The department shall prohibit a medicaid managed care
11 organization from (a) setting, establishing, or negotiating reimbursement
12 rates with a hospital in a manner that takes into account, directly or
13 indirectly, a directed payment that a hospital receives under the
14 Hospital Quality Assurance and Access Assessment Act, (b) unnecessarily
15 delaying a directed payment to a hospital, or (c) recouping or offsetting
16 a directed payment for any reason.

17 (7)(a) A hospital shall not:

18 (i) Set, establish, or negotiate reimbursement rates with a managed
19 care organization in a manner that directly or indirectly takes into
20 account a directed payment that a hospital receives under the Hospital
21 Quality Assurance and Access Assessment Act; or

22 (ii) Directly pass on the cost of an assessment to patients or
23 nonmedicaid payors, including as a fee or rate increase.

24 (b) A hospital that violates this subsection shall not receive a
25 directed payment for the remainder of the rate year. This subsection
26 shall not be construed to prohibit a hospital from negotiating with a
27 payor for a rate increase.

28 **Sec. 2.** Section 71-2454, Revised Statutes Cumulative Supplement,
29 2024, is amended to read:

30 71-2454 (1) A vendor ~~An entity~~ described in section 71-2455 shall
31 establish a system of prescription drug monitoring for the purposes of

1 (a) preventing the misuse of controlled substances that are prescribed,
2 (b) allowing prescribers and dispensers to monitor the care and treatment
3 of patients for whom such a prescription drug is prescribed to ensure
4 that such prescription drugs are used for medically appropriate purposes,
5 (c) providing information to improve the health and safety of patients,
6 and (d) ensuring that the State of Nebraska remains on the cutting edge
7 of medical information technology.

8 (2) Such system of prescription drug monitoring shall be implemented
9 as follows: Except as provided in subsection (4) of this section, all
10 prescription drug information shall be reported to the prescription drug
11 monitoring system. The prescription drug monitoring system shall include,
12 but not be limited to, provisions that:

13 (a) Prohibit any patient from opting out of the prescription drug
14 monitoring system;

15 (b) Require any prescription drug that is dispensed in this state or
16 to an address in this state to be entered into the system by the
17 dispenser or his or her delegate no less frequently than daily after such
18 prescription drug is sold, including prescription drugs for patients
19 paying cash or otherwise not relying on a third-party payor for payment,
20 except that prescriptions labeled "for emergency use" or "for use in
21 immunizations" are not required to be reported;

22 (c) Allow all prescribers or dispensers of prescription drugs to
23 access the system at no cost to such prescriber or dispenser;

24 (d) Ensure that such system includes information relating to all
25 payors, including, but not limited to, the medical assistance program
26 established pursuant to the Medical Assistance Act; and

27 (e) Make the prescription drug information available to the vendor
28 ~~statewide health information exchange~~ described in section 71-2455 for
29 access by its participants if such access is in compliance with the
30 privacy and security protections set forth in the provisions of the
31 federal Health Insurance Portability and Accountability Act of 1996,

1 Public Law 104-191, and regulations promulgated thereunder, except that
2 if a patient opts out of the ~~statewide~~ health information exchange
3 provided by the vendor, the prescription drug information regarding that
4 patient shall not be accessible by the participants in the ~~statewide~~
5 health information exchange provided by the vendor.

6 (3) Except as provided in subsection (4) of this section,
7 prescription drug information that shall be submitted electronically to
8 the prescription drug monitoring system shall be determined by the vendor
9 ~~entity~~ described in section 71-2455 and shall include, but not be limited
10 to:

11 (a) The patient's name, address, telephone number, if a telephone
12 number is available, gender, and date of birth;

13 (b) A patient identifier such as a military identification number,
14 driver's license number, state identification card number, or other valid
15 government-issued identification number, insurance identification number,
16 pharmacy software-generated patient-specific identifier, or other
17 identifier associated specifically with the patient;

18 (c) The name and address of the pharmacy dispensing the prescription
19 drug;

20 (d) The date the prescription is issued;

21 (e) The date the prescription is filled;

22 (f) The date the prescription is sold to the patient;

23 (g) The number of refills authorized;

24 (h) The prescription number of the prescription drug;

25 (i) The National Drug Code number as published by the federal Food
26 and Drug Administration of the prescription drug;

27 (j) The strength of the prescription drug prescribed;

28 (k) The quantity of the prescription drug prescribed and the number
29 of days' supply;

30 (l) The prescriber's name and National Provider Identifier number or
31 Drug Enforcement Administration number when reporting a controlled

1 substance; and

2 (m) Additional information as determined by the Health Information
3 Technology Board and as published in the submitter guide for the
4 prescription drug monitoring system.

5 (4) Beginning July 1, 2018, a veterinarian licensed under the
6 Veterinary Medicine and Surgery Practice Act shall be required to report
7 the dispensing of prescription drugs which are controlled substances
8 listed on Schedule II, Schedule III, Schedule IV, or Schedule V pursuant
9 to section 28-405. Each such veterinarian shall indicate that the
10 prescription is an animal prescription and shall include the following
11 information in such report:

12 (a) The first and last name and address, including city, state, and
13 zip code, of the individual to whom the prescription drug is dispensed in
14 accordance with a valid veterinarian-client-patient relationship;

15 (b) Reporting status;

16 (c) The first and last name of the prescribing veterinarian and his
17 or her federal Drug Enforcement Administration number;

18 (d) The National Drug Code number as published by the federal Food
19 and Drug Administration of the prescription drug and the prescription
20 number;

21 (e) The date the prescription is written and the date the
22 prescription is filled;

23 (f) The number of refills authorized, if any; and

24 (g) The quantity of the prescription drug and the number of days'
25 supply.

26 (5)(a) All prescription drug information submitted pursuant to this
27 section, all data contained in the prescription drug monitoring system,
28 and any report obtained from data contained in the prescription drug
29 monitoring system are confidential, are privileged, are not public
30 records, and may be withheld pursuant to section 84-712.05 except for
31 information released as provided in subsection (9) or (10) of this

1 section.

2 (b) No patient-identifying data as defined in section 81-664,
3 including the data collected under subsection (3) of this section, shall
4 be disclosed, made public, or released to any public or private person or
5 entity except to the vendor ~~statewide health information exchange~~
6 described in section 71-2455 and its participants, to prescribers and
7 dispensers as provided in subsection (2) of this section, or as provided
8 in subsection (7), (9), or (10) of this section.

9 (c) All other data is for the confidential use of the department and
10 the vendor ~~statewide health information exchange~~ described in section
11 71-2455 and its participants. The department, or the vendor ~~statewide~~
12 ~~health information exchange~~ in accordance with policies adopted by the
13 Health Information Technology Board and in collaboration with the
14 department, may release such information in accordance with the privacy
15 and security provisions set forth in the federal Health Insurance
16 Portability and Accountability Act of 1996, Public Law 104-191, and
17 regulations promulgated thereunder, as Class I, Class II, or Class IV
18 data in accordance with section 81-667, except for purposes in accordance
19 with subsection (9) or (10) of this section, to the private or public
20 persons or entities that the department or the vendor ~~statewide health~~
21 ~~information exchange~~, in accordance with policies adopted by the Health
22 Information Technology Board, determines may view such records as
23 provided in sections 81-663 to 81-675. In addition, the department, or
24 the vendor ~~statewide health information exchange~~ in accordance with
25 policies adopted by the Health Information Technology Board and in
26 collaboration with the department, may release such information as
27 provided in subsection (9) or (10) of this section.

28 (6) The vendor ~~statewide health information exchange~~ described in
29 section 71-2455, in accordance with policies adopted by the Health
30 Information Technology Board and in collaboration with the department,
31 shall establish the minimum administrative, physical, and technical

1 safeguards necessary to protect the confidentiality, integrity, and
2 availability of prescription drug information.

3 (7) If the entity receiving the prescription drug information has
4 privacy protections at least as restrictive as those set forth in this
5 section and has implemented and maintains the minimum safeguards required
6 by subsection (6) of this section, the vendor ~~statewide health~~
7 ~~information exchange~~ described in section 71-2455, in accordance with
8 policies adopted by the Health Information Technology Board and in
9 collaboration with the department, may release the prescription drug
10 information and any other data collected pursuant to this section to:

11 (a) Other state prescription drug monitoring programs;

12 (b) State and regional health information exchanges;

13 (c) The medical director and pharmacy director of the Division of
14 Medicaid and Long-Term Care of the department, or their designees;

15 (d) The medical directors and pharmacy directors of medicaid-managed
16 care entities, the state's medicaid drug utilization review board, and
17 any other state-administered health insurance program or its designee if
18 any such entities have a current data-sharing agreement with the vendor
19 ~~statewide health information exchange~~ described in section 71-2455, and
20 if such release is in accordance with the privacy and security provisions
21 of the federal Health Insurance Portability and Accountability Act of
22 1996, Public Law 104-191, and all regulations promulgated thereunder;

23 (e) Organizations which facilitate the interoperability and mutual
24 exchange of information among state prescription drug monitoring programs
25 or state or regional health information exchanges; or

26 (f) Electronic health record systems or pharmacy-dispensing software
27 systems for the purpose of integrating prescription drug information into
28 a patient's medical record.

29 (8) The department, or the vendor ~~statewide health information~~
30 ~~exchange~~ described in section 71-2455, in accordance with policies
31 adopted by the Health Information Technology Board and in collaboration

1 with the department, may release to patients their prescription drug
2 information collected pursuant to this section. Upon request of the
3 patient, such information may be released directly to the patient or a
4 personal health record system designated by the patient which has privacy
5 protections at least as restrictive as those set forth in this section
6 and that has implemented and maintains the minimum safeguards required by
7 subsection (6) of this section.

8 (9) In accordance with the privacy and security provisions set forth
9 in the federal Health Insurance Portability and Accountability Act of
10 1996, Public Law 104-191, and regulations promulgated thereunder, the
11 department, or the vendor ~~statewide health information exchange~~ described
12 in section 71-2455 under policies adopted by the Health Information
13 Technology Board, may release data collected pursuant to this section for
14 statistical, public policy, or educational purposes after removing
15 information which identifies or could reasonably be used to identify the
16 patient, prescriber, dispenser, or other person who is the subject of the
17 information, except as otherwise provided in subsection (10) of this
18 section.

19 (10) In accordance with the privacy and security provisions set
20 forth in the federal Health Insurance Portability and Accountability Act
21 of 1996, Public Law 104-191, and regulations promulgated thereunder, the
22 department, or vendor ~~statewide health information exchange~~ described in
23 section 71-2455 under policies adopted by the Health Information
24 Technology Board, may release data collected pursuant to this section for
25 quality measures as approved or regulated by state or federal agencies or
26 for patient quality improvement or research initiatives approved by the
27 Health Information Technology Board.

28 (11) The vendor ~~statewide health information exchange~~ described in
29 section 71-2455, entities described in subsection (7) of this section, or
30 the department may request and receive program information from other
31 prescription drug monitoring programs for use in the prescription drug

1 monitoring system in this state in accordance with the privacy and
2 security provisions set forth in the federal Health Insurance Portability
3 and Accountability Act of 1996, Public Law 104-191, and regulations
4 promulgated thereunder.

5 (12) The vendor ~~statewide health information exchange~~ described in
6 section 71-2455, in collaboration with the department, shall implement
7 technological improvements to facilitate the secure collection of, and
8 access to, prescription drug information in accordance with this section.

9 (13) Before accessing the prescription drug monitoring system, any
10 user shall undergo training on the purpose of the system, access to and
11 proper usage of the system, and the law relating to the system, including
12 confidentiality and security of the prescription drug monitoring system.
13 Such training shall be administered by the vendor ~~statewide health~~
14 ~~information exchange~~ described in section 71-2455 or the department. The
15 vendor ~~statewide health information exchange~~ described in section 71-2455
16 shall have access to the prescription drug monitoring system for training
17 operations, maintenance, and administrative purposes. Users who have been
18 trained prior to May 10, 2017, or who are granted access by an entity
19 receiving prescription drug information pursuant to subsection (7) of
20 this section, are deemed to be in compliance with the training
21 requirement of this subsection.

22 (14) For purposes of this section:

23 (a) Deliver or delivery means to actually, constructively, or
24 attempt to transfer a drug or device from one person to another, whether
25 or not for consideration;

26 (b) Department means the Department of Health and Human Services;

27 (c) Delegate means any licensed or registered health care
28 professional credentialed under the Uniform Credentialing Act designated
29 by a prescriber or dispenser to act as an agent of the prescriber or
30 dispenser for purposes of submitting or accessing data in the
31 prescription drug monitoring system and who is supervised by such

1 prescriber or dispenser;

2 (d) Prescription drug or drugs means a prescription drug or drugs
3 dispensed by delivery to the ultimate user or caregiver by or pursuant to
4 the lawful order of a prescriber but does not include (i) the delivery of
5 such prescription drug for immediate use for purposes of inpatient
6 hospital care or emergency department care, (ii) the administration of a
7 prescription drug by an authorized person upon the lawful order of a
8 prescriber, (iii) a wholesale distributor of a prescription drug
9 monitored by the prescription drug monitoring system, or (iv) the
10 dispensing to a nonhuman patient of a prescription drug which is not a
11 controlled substance listed in Schedule II, Schedule III, Schedule IV, or
12 Schedule V of section 28-405;

13 (e) Dispenser means a person authorized in the jurisdiction in which
14 he or she is practicing to deliver a prescription drug to the ultimate
15 user or caregiver by or pursuant to the lawful order of a prescriber;

16 (f) Participant means an individual or entity that has entered into
17 a participation agreement with the vendor ~~statewide health information~~
18 ~~exchange~~ described in section 71-2455 which requires the individual or
19 entity to comply with the privacy and security protections set forth in
20 the provisions of the federal Health Insurance Portability and
21 Accountability Act of 1996, Public Law 104-191, and regulations
22 promulgated thereunder; and

23 (g) Prescriber means a health care professional authorized to
24 prescribe in the profession which he or she practices.

25 **Sec. 3.** Section 71-2455, Revised Statutes Cumulative Supplement,
26 2024, is amended to read:

27 71-2455 Subject to sections 81-6,127 and 81-6,128, the Department of
28 Health and Human Services shall contract with a vendor pursuant to the
29 State Procurement Act to , ~~in collaboration with the Nebraska Health~~
30 ~~Information Initiative or any successor public-private statewide health~~
31 ~~information exchange,~~ shall enhance or establish technology for

1 prescription drug monitoring to carry out the purposes of section
2 71-2454. The department may use state funds and accept grants, gifts, or
3 other funds in order to implement and operate the technology. The
4 department may adopt and promulgate rules and regulations to authorize
5 use of electronic health information, if necessary to carry out the
6 purposes of sections 71-2454 and 71-2455. The department shall contract
7 with a vendor pursuant to the State Procurement Act ~~the statewide health~~
8 ~~information exchange~~ for the administration of the Health Information
9 Technology Board, and such contract shall specify that the vendor ~~health~~
10 ~~information exchange~~ is responsible for the administration of the Health
11 Information Technology Board, including, but not limited to, providing
12 meeting notices, recording and distributing meeting minutes,
13 administrative tasks related to the same, and funding such activities.
14 The contract shall also include provisions for the vendor ~~statewide~~
15 ~~health information exchange~~ to reimburse the expenses of the members of
16 the board pursuant to subsection (5) of section 81-6,127. Such
17 reimbursement shall be paid using a process essentially similar to
18 sections 81-1174 to 81-1177. No state funds, including General Funds,
19 cash funds, and federal funds, shall be used to carry out the
20 administrative duties of the Health Information Technology Board nor for
21 reimbursement of the expenses of the board members.

22 **Sec. 4.** Section 81-6,124, Reissue Revised Statutes of Nebraska, is
23 amended to read:

24 81-6,124 For purposes of the Population Health Information Act:

25 (1) Clinical information means information related to the diagnosis
26 and treatment of health conditions or services provided for health
27 conditions;

28 (2) Department means the Department of Health and Human Services;

29 (3) Designated health information exchange means the vendor
30 ~~statewide health information exchange~~ described in section 71-2455;

31 (4) Health care entity means a health care facility as defined in

1 section 71-413, a home health agency as defined in section 71-417, an
2 urgent care treatment center, a laboratory, a medicaid managed care
3 organization, a federally qualified health center, a health care
4 practitioner facility as defined in section 71-414, a dental facility, a
5 local public health department, a health insurance carrier, or any other
6 organization or entity providing health care services in Nebraska;

7 (5) Health care provider means a person practicing as a health care
8 professional under the Uniform Credentialing Act; and

9 (6) Prescription drug monitoring program means the program created
10 under section 71-2454.

11 **Sec. 5.** Section 81-6,125, Reissue Revised Statutes of Nebraska, is
12 amended to read:

13 81-6,125 (1) The purpose of the Population Health Information Act is
14 to designate a vendor ~~health information exchange~~ to provide the data
15 infrastructure needed to assist in creating a healthier Nebraska and
16 operating the electronic health records initiative.

17 (2) The designated health information exchange shall:

18 (a) Aggregate clinical information from health care entities needed
19 to support the operation of the medical assistance program under the
20 Medical Assistance Act;

21 (b) Act as the designated entity for purposes of access to and
22 analysis of health data;

23 (c) Collect and analyze data for purposes of informing the
24 Legislature, the department, health care providers, and health care
25 entities as to the cost of, access to, and quality of health care in
26 Nebraska;

27 (d) Act as a collector and reporter of public health data for
28 registry submissions, electronic laboratory reporting, immunization
29 reporting, and syndromic surveillance from an electronic health record,
30 which does not include claims data; and

31 (e) Enable any health care provider or health care entity to access

1 information available within the designated health information exchange
2 to evaluate and monitor care and treatment of a patient in accordance
3 with the privacy and security provisions set forth in the federal Health
4 Insurance Portability and Accountability Act of 1996, Public Law 104-191.

5 (3)(a) On or before September 30, 2021, each health care facility
6 listed in subdivision (b) of this subsection shall participate in the
7 designated health information exchange through sharing of clinical
8 information. Such clinical information shall include the clinical data
9 that the health care facility captured in its existing electronic health
10 record as permitted by state and federal laws, rules, and regulations.
11 Any patient health information shared with the designated health
12 information exchange as determined by policies adopted by the Health
13 Information Technology Board shall be provided in accordance with the
14 privacy and security provisions set forth in the federal Health Insurance
15 Portability and Accountability Act of 1996 and regulations adopted under
16 the act.

17 (b) This subsection applies to an ambulatory surgical center, a
18 critical access hospital, a general acute hospital, a health clinic, a
19 hospital, an intermediate care facility, a long-term care hospital, a
20 mental health substance use treatment center, a PACE center, a pharmacy,
21 a psychiatric or mental hospital, a public health clinic, or a
22 rehabilitation hospital, as such terms are defined in the Health Care
23 Facility Licensure Act, or a diagnostic, laboratory, or imaging center.

24 (c) This subsection does not apply to (i) a state-owned or state-
25 operated facility or (ii) an assisted-living facility, a nursing
26 facility, or a skilled nursing facility, as such terms are defined in the
27 Health Care Facility Licensure Act.

28 (d) Any connection established by July 1, 2021, between a health
29 care facility and the designated health information exchange to
30 facilitate such participation shall be at no cost to the participating
31 health care facility.

1 (e) A health care facility may apply to the board for a waiver from
2 the requirement to participate under this subsection due to a
3 technological burden. The board shall review the application and
4 determine whether to waive the requirement. If the board waives the
5 requirement for a health care facility, the board shall review the waiver
6 annually to determine if the health care facility continues to qualify
7 for the waiver.

8 (f) The board shall not require a health care facility to purchase
9 or contract for an electronic records management system or service.

10 (4)(a) On or before January 1, 2022, each health insurance plan
11 shall participate in the designated health information exchange through
12 sharing of information. Subject to subsection (5) of this section, such
13 information shall be determined by policies adopted by the Health
14 Information Technology Board.

15 (b) For purposes of this subsection:

16 (i) Health insurance plan includes any group or individual sickness
17 and accident insurance policy, health maintenance organization contract,
18 subscriber contract, employee medical, surgical, or hospital care benefit
19 plan, or self-funded employee benefit plan to the extent not preempted by
20 federal law; and

21 (ii) Health insurance plan does not include (A) accident-only,
22 disability-income, hospital confinement indemnity, dental, hearing,
23 vision, or credit insurance, (B) coverage issued as a supplement to
24 liability insurance, (C) insurance provided as a supplement to medicare,
25 (D) insurance arising from workers' compensation provisions, (E)
26 automobile medical payment insurance, (F) insurance policies that provide
27 coverage for a specified disease or any other limited benefit coverage,
28 or (G) insurance under which benefits are payable with or without regard
29 to fault and which is statutorily required to be contained in any
30 liability insurance policy.

31 (5) The designated health information exchange and the department

1 shall enter into an agreement to allow the designated health information
2 exchange to collect, aggregate, analyze, report, and release de-
3 identified data, as defined by the federal Health Insurance Portability
4 and Accountability Act of 1996, that is derived from the administration
5 of the medical assistance program. Such written agreement shall be
6 executed no later than September 30, 2021.

7 (6) In addition to the right to opt out as provided in section
8 71-2454, an individual shall have the right to opt out of the designated
9 health information exchange or the sharing of information required under
10 subsections (3) and (4) of this section. The designated health
11 information exchange shall adopt a patient opt-out policy consistent with
12 the federal Health Insurance Portability and Accountability Act of 1996
13 and other applicable federal requirements. Such policy shall not apply to
14 mandatory public health reporting requirements.

15 **Sec. 6.** Section 81-6,127, Reissue Revised Statutes of Nebraska, is
16 amended to read:

17 81-6,127 (1) The Health Information Technology Board is created. The
18 board shall have seventeen members. Except for members designated in
19 subdivision (2)(o) of this section, the members shall be appointed by the
20 Governor with the approval of a majority of the members of the
21 Legislature. The members may begin to serve immediately following
22 appointment and prior to approval by the Legislature. The members shall
23 be appointed by February 1, 2021, and the board shall begin meeting on or
24 before April 1, 2021.

25 (2) Members designated under subdivisions (b), (c), (d), (e), (g),
26 (h), and (i) of this subsection shall hold a credential under the Uniform
27 Credentialing Act. Except as otherwise provided in subsection (4) of this
28 section, the board shall consist of:

29 (a) One individual who has experience in operating the prescription
30 drug monitoring program created under section 71-2454;

31 (b) Two physicians, one of whom shall be a family practice

1 physician, who are in active practice and in good standing with the
2 Department of Health and Human Services appointed from a list of
3 physicians provided by a statewide organization representing physicians;

4 (c) One pharmacist who is in active practice and in good standing
5 with the department appointed from a list of pharmacists provided by a
6 statewide organization representing pharmacists;

7 (d) One alcohol and drug counselor providing services for a state-
8 licensed alcohol and drug abuse addiction treatment program;

9 (e) One health care provider who is board-certified in pain
10 management;

11 (f) One hospital administrator appointed from a list of hospital
12 administrators provided by a statewide organization representing hospital
13 administrators;

14 (g) One dentist who is in active practice and in good standing with
15 the department appointed from a list of dentists provided by a statewide
16 organization representing dentists;

17 (h) One nurse practitioner who is in active practice and in good
18 standing with the department authorized to prescribe medication appointed
19 from a list of nurse practitioners authorized to prescribe medication
20 provided by a statewide organization representing such nurse
21 practitioners;

22 (i) One veterinarian who is in active practice and in good standing
23 with the department appointed from a list of veterinarians provided by a
24 statewide organization representing veterinarians;

25 (j) One representative of the Department of Health and Human
26 Services;

27 (k) One representative of a delegate as defined in section 71-2454;

28 (l) One health care payor as defined in section 25-21,247 or an
29 employee of a health care payor;

30 (m) One credentialed health information management professional
31 appointed from a list of such professionals provided by a statewide

1 organization representing such professionals;

2 (n) One representative of the vendor ~~statewide health information~~
3 ~~exchange~~ described in section 71-2455; and

4 (o) The chairperson of the Health and Human Services Committee of
5 the Legislature and the chairperson of the Appropriations Committee of
6 the Legislature, both of whom are nonvoting, ex officio members.

7 (3) Except for members designated in subdivisions (2)(a) and (o) of
8 this section:

9 (a) A minimum of three members shall be appointed from each
10 congressional district;

11 (b) Each member shall be appointed for a five-year term beginning on
12 April 1, 2021, and may serve for any number of such terms;

13 (c) Any member appointed prior to April 1, 2021, shall begin to
14 serve immediately upon appointment and continue serving for the term
15 beginning on April 1, 2021; and

16 (d) Any vacancy in membership, other than by expiration of a term,
17 shall be filled within ninety days by the Governor by appointment for the
18 vacant position as provided in subsection (2) of this section.

19 (4) If, after appointment, the classification of a member's
20 credential changes or a member's credential classification is terminated
21 and if such credential was a qualification for appointment, the member
22 shall be permitted to continue to serve as a member of the board until
23 the expiration of the term for which appointed unless the member loses
24 the credential due to disciplinary action.

25 (5) The members shall be reimbursed for their actual and necessary
26 expenses incurred in serving on the board as provided in section 71-2455.

27 (6) A simple majority of members shall constitute a quorum for the
28 transaction of all business.

29 **Sec. 7.** Section 81-6,128, Reissue Revised Statutes of Nebraska, is
30 amended to read:

31 81-6,128 (1) The Health Information Technology Board shall:

1 (a) Establish criteria for data collection and disbursement by the
2 ~~vendor statewide health information exchange~~ described in section 71-2455
3 and the prescription drug monitoring program created under section
4 71-2454 to improve the quality of information provided to clinicians;

5 (b) Evaluate and ensure that the statewide health information
6 exchange is meeting technological standards for reporting of data for the
7 prescription drug monitoring program, including the data to be collected
8 and reported and the frequency of data collection and disbursement;

9 (c) Provide the governance oversight necessary to ensure that any
10 health information in the statewide health information exchange and the
11 prescription drug monitoring program may be accessed, used, or disclosed
12 only in accordance with the privacy and security protections set forth in
13 the federal Health Insurance Portability and Accountability Act of 1996,
14 Public Law 104-191, and regulations promulgated thereunder. All protected
15 health information is privileged, is not a public record, and may be
16 withheld from the public pursuant to section 84-712.05; and

17 (d) Provide recommendations to the statewide health information
18 exchange on any other matters referred to the board.

19 (2) The board shall adopt policies and procedures necessary to carry
20 out its duties.

21 (3) The authority of the board to direct the use or release of data
22 under this section or section 71-2454 shall apply only to requests
23 submitted to the board after September 1, 2021.

24 (4) The board may hold meetings by telecommunication or electronic
25 communication subject to the Open Meetings Act. Any official action or
26 vote of the members of the board shall be preserved in the records of the
27 board.

28 (5) By November 15, 2021, and November 15 of each year thereafter,
29 the board shall develop and submit an annual report to the Governor and
30 the Health and Human Services Committee of the Legislature regarding
31 considerations undertaken, decisions made, accomplishments, and other

1 relevant information. The report submitted to the Legislature shall be
2 submitted electronically.

3 **Sec. 8.** Original sections 81-6,124, 81-6,125, 81-6,127, and
4 81-6,128, Reissue Revised Statutes of Nebraska, and sections 68-2106,
5 71-2454, and 71-2455, Revised Statutes Cumulative Supplement, 2024, are
6 repealed.